

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Thomas Greene
Date of birth	February 14, 1956
Age at visit	70 years
Sex	Male
MRN	CAR-23011
Address	18 South Plant Avenue, Tampa, FL 33606
Phone	(813) 555-0124
Emergency contact	Marie Greene (spouse), (813) 555-0125
Primary care	Dr. Carl Brooks, MD, (813) 555-0143
Preferred language	English

2. Insurance

Primary insurance

Payer	Palm Mutual of Florida, Medicare Advantage
Member ID	PMMA552810
Group	PM-MA-09
Prior authorization	Required for carotid surgery
Auth telephone	1-800-555-0177

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I65.22	Occlusion and stenosis of left carotid artery	2026	Active
G45.3	Amaurosis fugax, left eye	2026	Active
I10	Essential hypertension	2006	Active
E78.5	Hyperlipidemia, unspecified	2011	Active

4. Current medications

Medication	Dose	Frequency	Indication
Clopidogrel	75 mg	Daily	Antiplatelet after amaurosis
Atorvastatin	40 mg	Daily	Lipids

Medication	Dose	Frequency	Indication
Metoprolol succinate	25 mg	Daily	Hypertension

5. Allergies

Allergen	Reaction	Severity
No known drug allergies	Not applicable	Not applicable

6. Visit notes — vascular surgery

Visit date: March 11, 2026 | **Attending:** Owen Clarke, MD, FACS | **Type:** New patient consult

Chief complaint. Two episodes of left-eye vision loss in February 2026.

History of present illness. Thomas Greene is a 70-year-old retired plumber. On February 2, 2026 and again on February 18, 2026 he had a gray curtain descend over the left eye for about five minutes, then clear completely. Ophthalmology found no retinal detachment and diagnosed amaurosis fugax. He has had no arm or leg weakness and no speech change. Examination today is normal. He has not had a myocardial infarction. He prefers an incision and a patch to a stent. He does not have leg pain, and this visit is not for peripheral artery disease.

Examination.

Blood pressure 142/76 mmHg	Neurologic exam Normal, including visual fields today
Eyes No current vision loss. Episodes were transient.	Heart Regular rhythm. No recent chest pain.
Legs No claudication. Pedal pulses present. This is not a leg-artery request.	

Assessment and plan.

Left-eye amaurosis fugax on February 2, 2026 and February 18, 2026, ipsilateral to a left internal carotid stenosis, within six months of this March 11, 2026 request.

CTA February 25, 2026: 85 percent NASCET stenosis of the left cervical internal carotid artery, not occluded.

Plan: left carotid endarterectomy by neck incision with patch angioplasty, CPT 35301. A stent is not ordered.

Electronically signed: Owen Clarke, MD, FACS — March 11, 2026 at 9:55 AM

7. Diagnostic reports

CTA neck — February 25, 2026

Indication	Left-eye amaurosis fugax
Measurement	NASCET
Date	February 25, 2026, within six months of March 11, 2026

Interpretation. Left cervical internal carotid artery: 85 percent stenosis by NASCET criteria. The distal internal carotid artery is patent. The artery is not occluded. The plaque is extracranial. Right internal carotid artery: 20 percent by NASCET. No intracranial occlusion. Impression: 85 percent NASCET stenosis of the left cervical internal carotid artery, not occluded, ipsilateral to left-eye amaurosis fugax.

Electronically signed: Bayshore Imaging — February 25, 2026

8. Planned procedure

Planned procedure	Left carotid endarterectomy with patch angioplasty
CPT	35301 — thromboendarterectomy, carotid, by neck incision

Laterality	Left cervical internal carotid artery
Not ordered	No carotid stent. No leg angiogram. No femoropopliteal stent.
Surgeon	Owen Clarke, MD, FACS
Payer	Palm Mutual of Florida Medicare Advantage
Authorization status	Not submitted. Request dated March 11, 2026.

Clinical indication summary

Left-eye amaurosis fugax on February 2, 2026 and February 18, 2026.

CTA February 25, 2026: 85 percent NASCET stenosis of the left cervical internal carotid artery. The artery is not occluded.

Ordered procedure is carotid endarterectomy, CPT 35301, not a stent and not a leg revascularization.

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